

OPPORTUNITY REVIEW

DIRECT-TO- EMPLOYER GTM STRATEGY

Exploring a Franciscan Missionaries of Our Lady Health System
DTE Health Plan

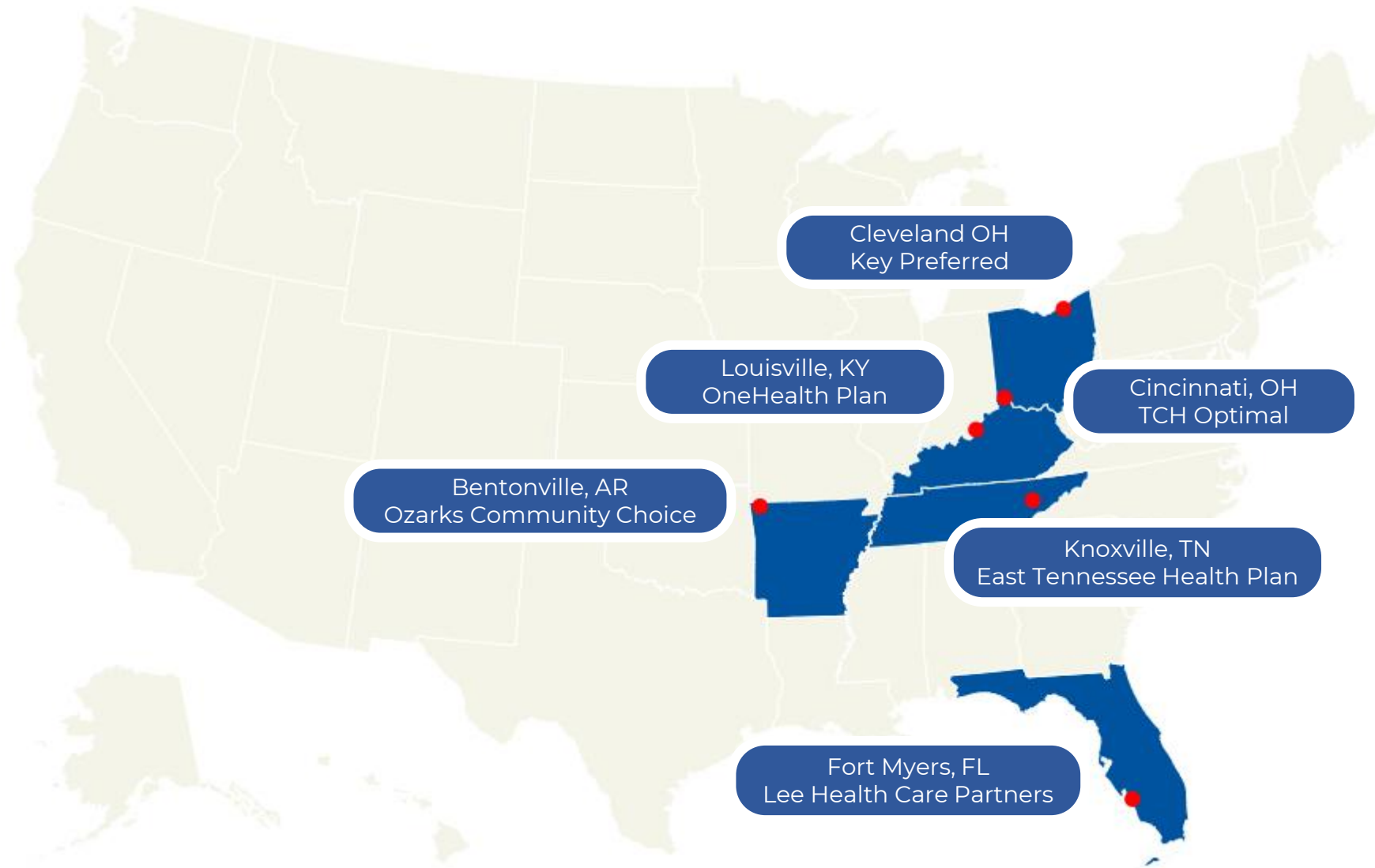


An aerial photograph of the Indianapolis skyline at sunset. The Indiana State Capitol is prominent on the left, featuring the Soldiers and Sailors Monument. The city is filled with various buildings, including modern glass-fronted structures and older brick buildings. The sky is a mix of orange, yellow, and blue, indicating the time is either sunrise or sunset. The overall scene is a wide-angle shot looking down on the city from a high vantage point.

KEY BENEFIT ADMINISTRATORS

- Founded in 1979, headquartered in Indianapolis
- Largest Independently-Owned Full-Service Third-Party Administrator
- Specialize in Administration of Self-Funded Plans and Population Health Management
- Leader in the Direct-to-Employer Movement
- We Know and Understand the Market
- Experience Working with All Professional Teams: Agents, Networks, MGAs
- The Key Family of Companies is a Vertically Integrated group of benefit related organizations
 - 3,000+ Corporate Clients
 - 3 Million Members under Management
 - 700 Employees

MARKET LEADER IN PRIVATE LABELED DTE PLANS





Our Founding and Heritage:

46 Years of Striving for Honesty in Healthcare

Founded Key Benefit Administrators in 1979

After co-founding a life insurance agency in 1973 and Key Life Insurance in 1976, Larry Dust launched KBA in 1979 as a third-party benefits administrator focused on honest healthcare delivery.

Sold to Blue Cross/Blue Shield in 1985

Strategic partnership with BCBS of Indiana (then part of the Associated Group) provided capital and market access while maintaining operational independence.

Reacquired KBA in 1992

Larry bought the business back from BCBS in 1992, returning it to complete independence and patient-focused mission alignment.

Built the Largest Independent TPA in the U.S.

Under his leadership since the buyback, KBA has become the country's largest independently owned third-party administrator, managing millions of members across diverse markets.

Never Forgotten Who Gives Healthcare

KBA has never participated in reference-based pricing, does not take refunds, and delivers healthcare at 11% below market rates. Our commitment remains with providers and patients, not profit extraction.

Key Benefit Administrators

DIRECT-TO-EMPLOYER (DTE)

Your Health Plan
Focused on a Direct
Relationship with Self-
Funded Employers in
Your Market



YOU Pick the Name of the Health Plan



YOU Determine the Fees Charged



YOU Practice the Medicine



YOU Receive the Profits



WE Make it Successful

DTE KEYS TO SUCCESS FOR FMOLHS

1

DTE Plan Sells

DTE plans only work when the network is built around a strong brand with good access and competitive costs.

Franciscan Missionaries of Our Lady Health System is the quality and access leader in this service area (see slide 8). Franciscan Missionaries of Our Lady Health System LA has a very strong quality brand, but market size will limit plan growth

2

Estimates for Margin and Steerage Hold

KBA models high, medium and low estimates for net income based on (1) estimates of commercial margin and (2) the within-DTE-network steerage rate.

Profit growth will hold if these assumptions accurately play out as expected for Franciscan Missionaries of Our Lady Health System.



Direct-to-Employer Strategy: **POWERFUL EMPLOYER VALUE**

- Reduced medical spend through lower unit costs and better integrated care
- “At the table” with Franciscan Missionaries of Our Lady Health System
- Aligned financial incentives
- Complete transparency for unlimited visibility into health benefit plan expenses and cost drivers
- Aggressive stop-loss premiums and policy terms
- Increased employee satisfaction
- Improved cash flow & stabilization of future medical cost trend

Proof: 98% of Our Clients Beat Healthcare Inflation

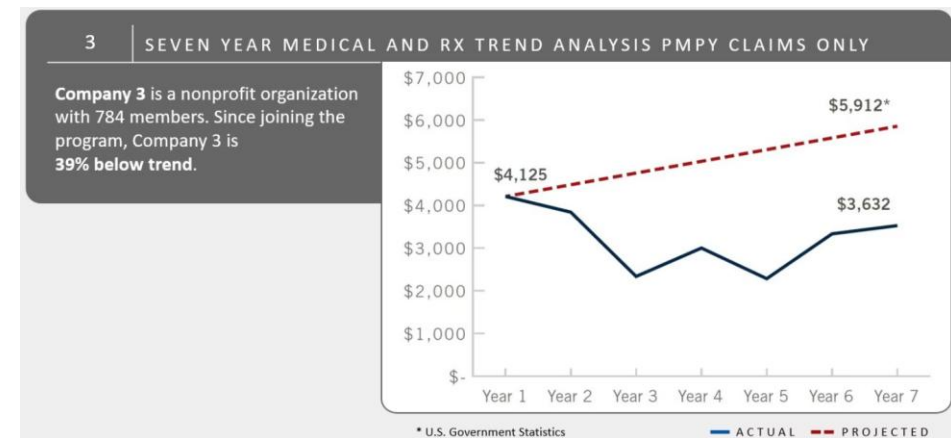
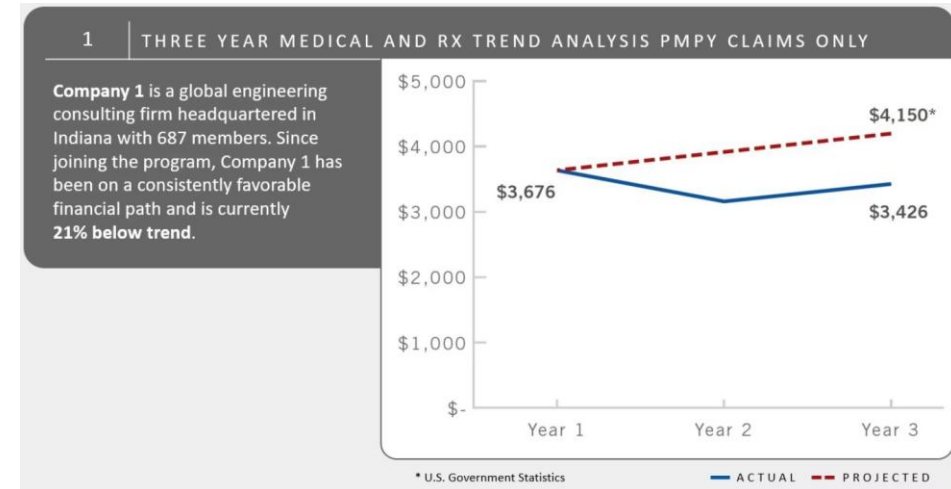
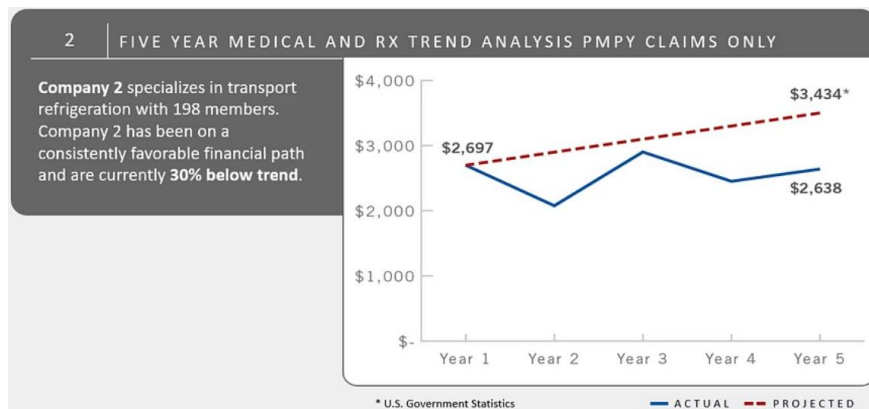
The Red Line = Healthcare CPI

The year-over-year healthcare component of the consumer price index represents typical market healthcare cost increases that employers face nationwide.

The Blue Line = Main Street Management

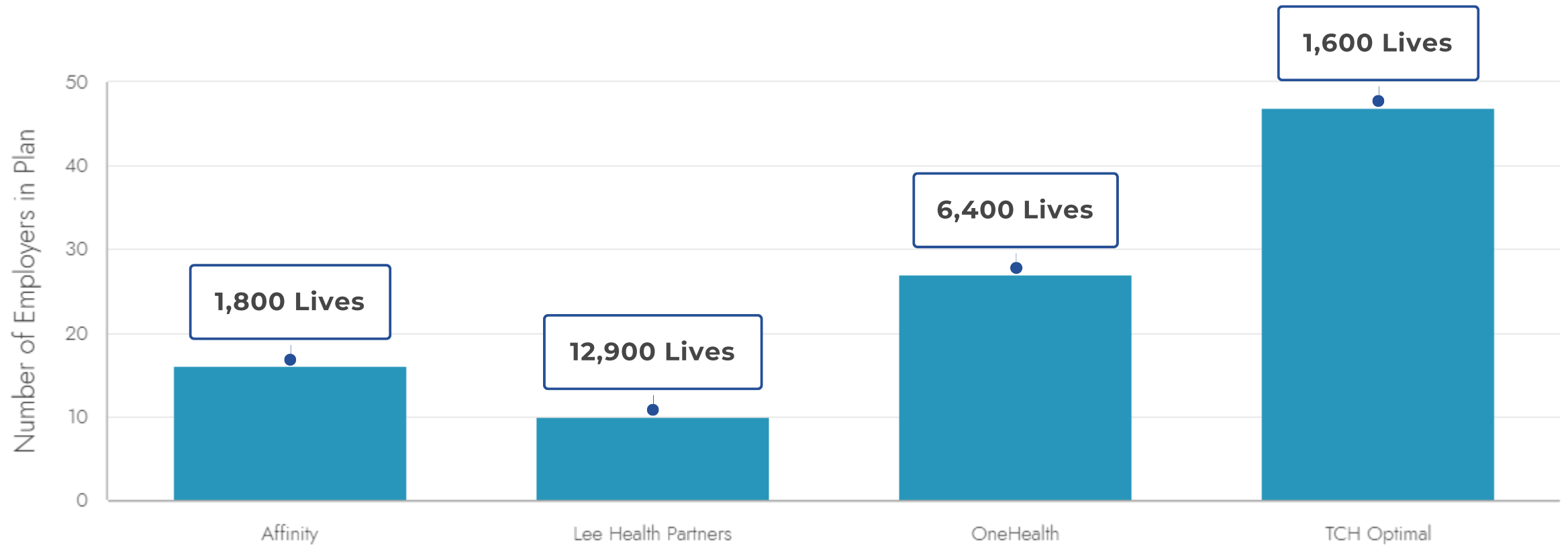
What happens when you manage your health on Main Street with direct provider relationships, transparent pricing, and aligned incentives.

For our clients, 98% of our Blue Lines are BELOW their Red Lines!



This consistent outperformance demonstrates that direct-to-employer partnerships deliver measurable, sustainable value for all stakeholders involved.

KBA PLAN GROWTH IN THE FIRST TWO YEARS



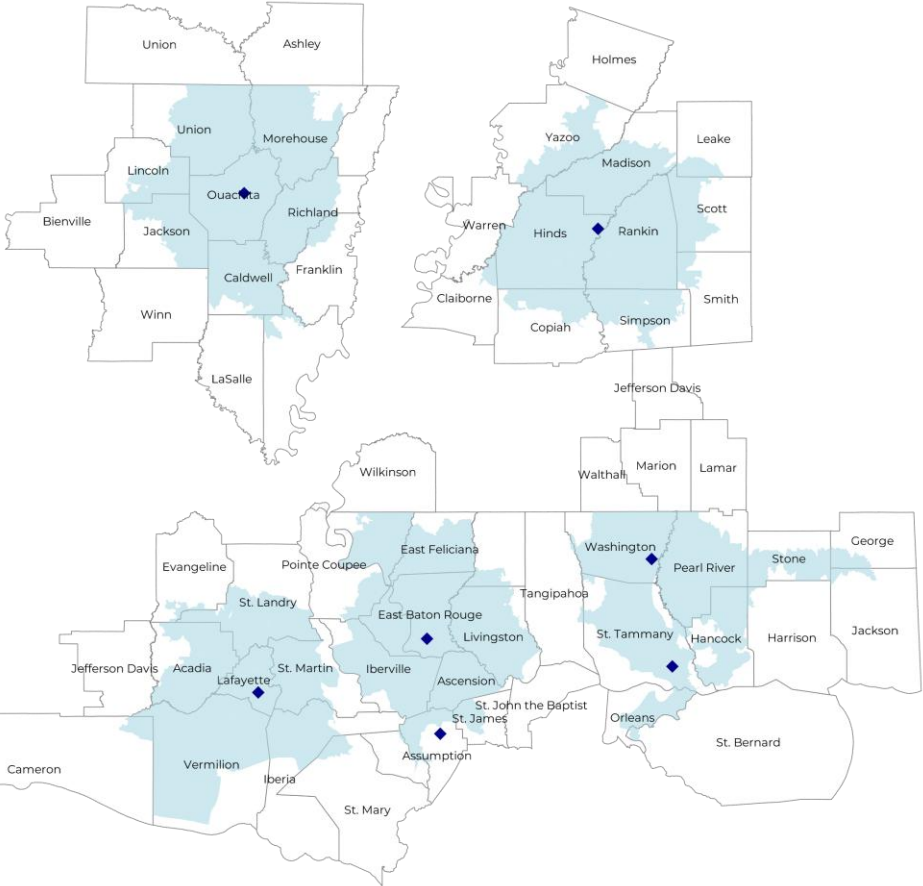
An average of 25 employer groups in plan within the first two years!

SIGNIFICANT OPPORTUNITY FOR PLAN GROWTH

Researched for the below highlighted service area

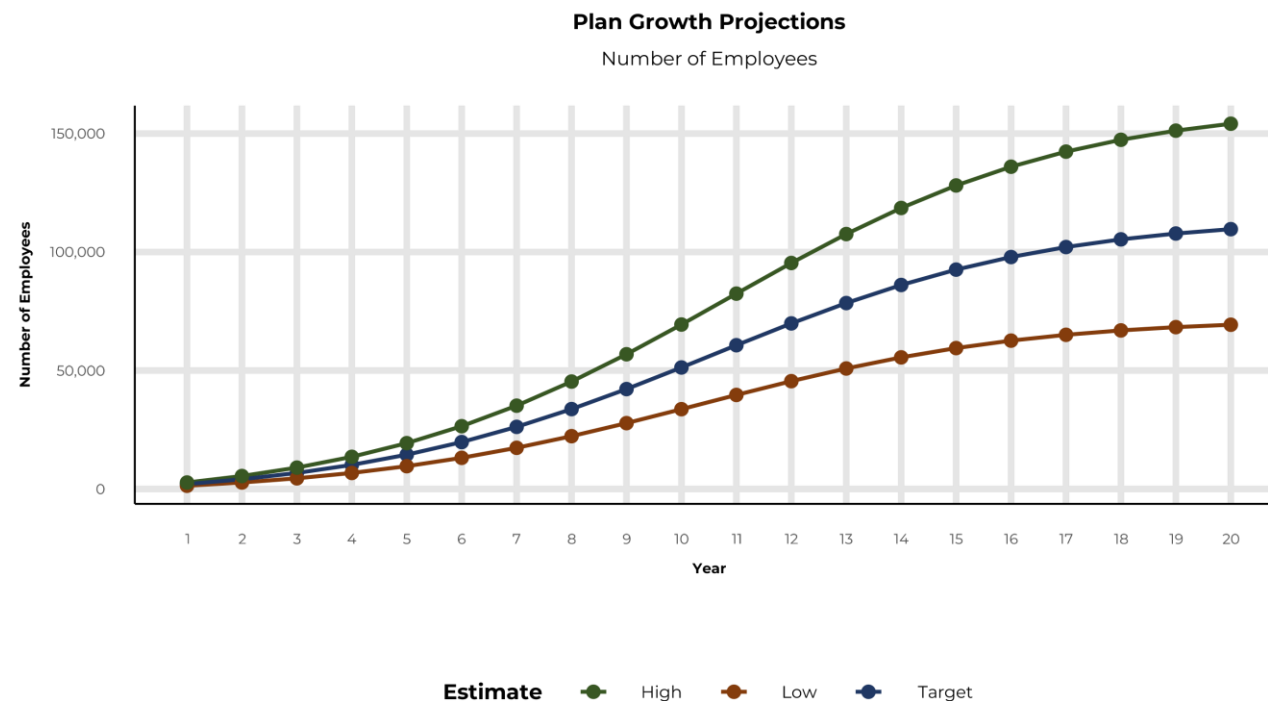
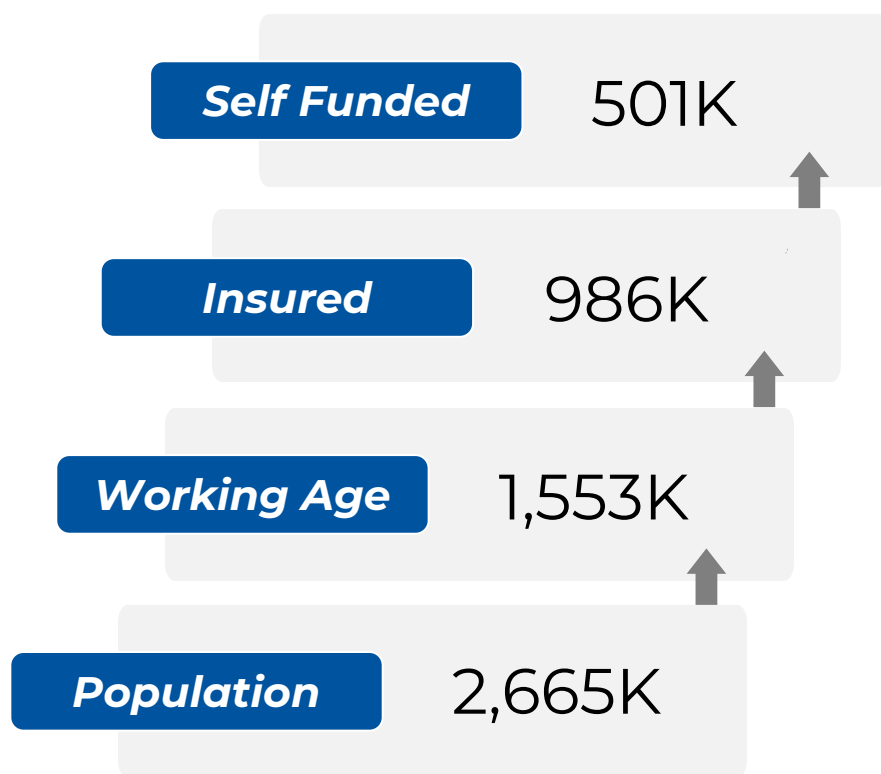
Target Market

2,500+ Employers ➤ 500,000+ Employees ➤ 900,00+ Members



	Employer Size Group (# of Employees)					
	50-100	101-200	201-400	401-1,000	1,001-3,000	3,000+
Commercial Employers With Headquarters In Catchment Area	1,309	608	343	205	60	33
Total Employees at Employers Headquartered in Catchment Area	98,200	91,200	102,900	143,500	120,000	198,000
Total Members with Employers Headquartered in Catchment Area	176,800	164,200	185,200	258,300	216,000	356,400

PLAN POTENTIAL GROWTH PATHS



Estimates includes the Franciscan Missionaries of Our Lady Health System service area in Acadia, Ascension, Assumption, Caldwell, East Baton Rouge, East Feliciana, Franklin, Iberia, Iberville, Jackson, Lafayette, Lincoln, Livingston, Morehouse, Orleans, Ouachita, Pointe Coupee, Richland, St. Bernard, St. Helena, St. James, St. Landry, St. Martin, St. Tammany, Union, Vermilion, Washington, West Baton Rouge, West Feliciana, Copiah, Hancock, Hinds, Leake, Madison, Pearl River, Rankin, Scott, Simpson, Stone, and Yazoo Counties

GROWING POWERFUL, LASTING PROFITABILITY

Early Stage

Gaining Name Recognition, Early Growth

New Revenue: **\$55.03M** New Profit: **\$18.52M**

Growth Stage

Strong Reputation, Strong Growth

New Revenue: **\$327.34M** New Profit: **\$111.05M**

Mature Stage

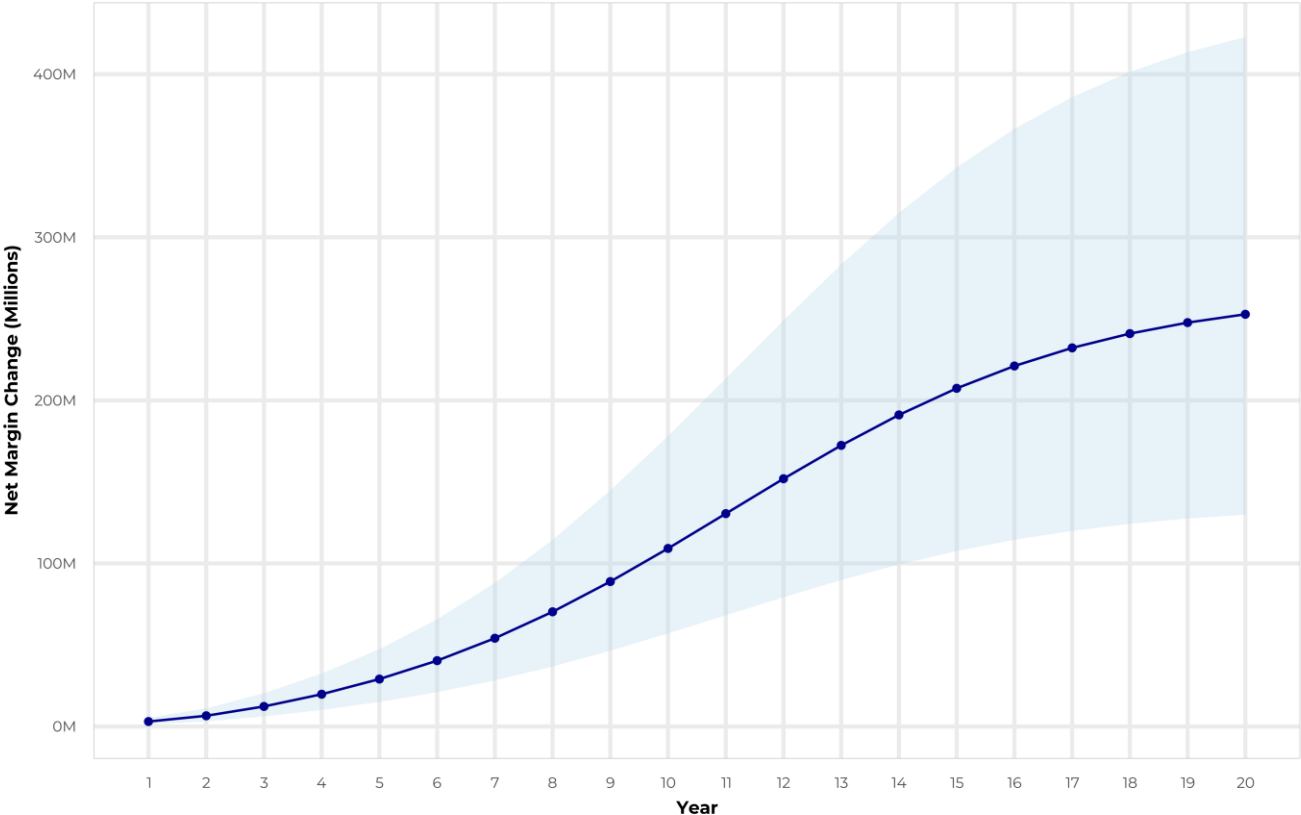
High Profitability, Market Saturation

New Revenue: **\$670.71M** New Profit: **\$227.58M**

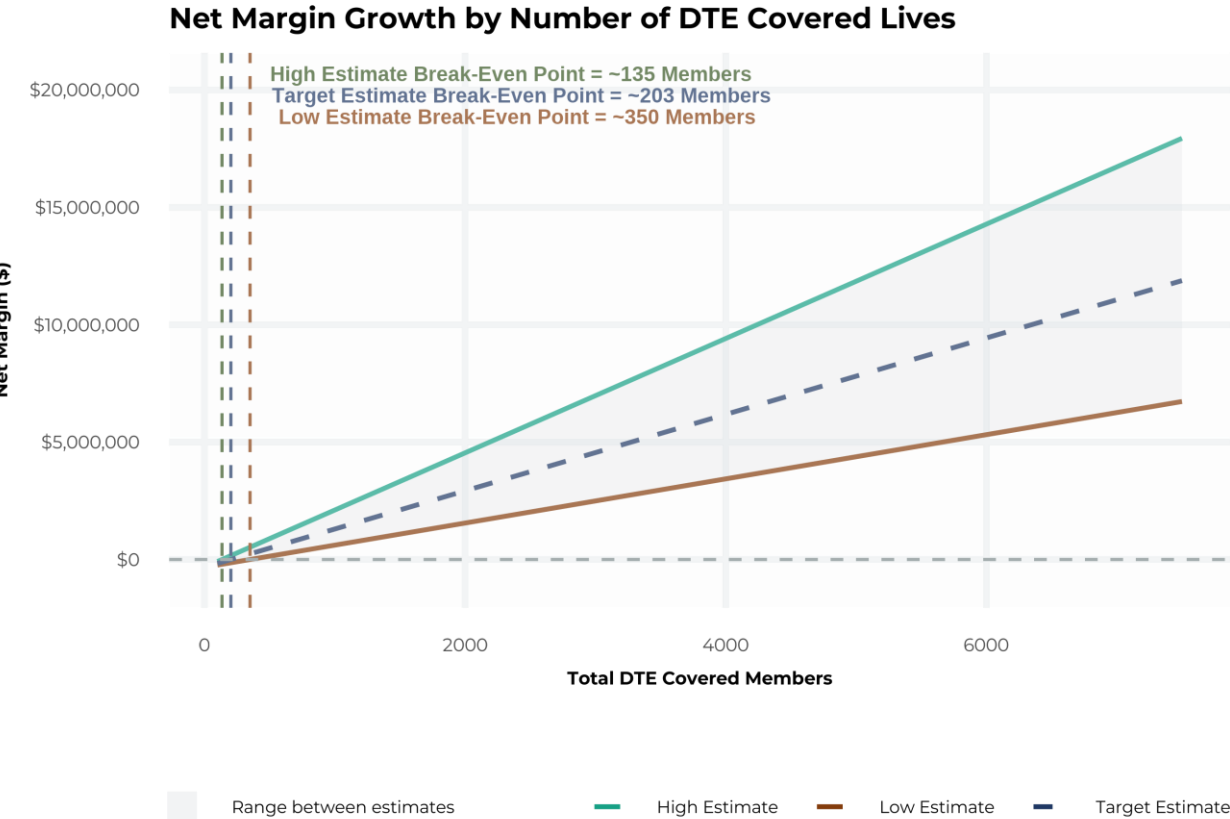
Net Margin Change Over Years

Variation by Member Growth With Target Estimate Assuming

50%+ Within-Plan Steerage and a Commercial Margin of 33.6%



MODEL ASSUMPTIONS AND BREAK EVEN



Patient Steerage Assumptions

KBA currently estimates that Franciscan Missionaries of Our Lady Health System has 25% inpatient market share, and 10% outpatient market compared to hospitals/clinics in the Franciscan Missionaries of Our Lady Health System service area (see slide 8). The DTE tiered structure will create immediate steerage, with additional steerage over time from patient bonus benefits and the patient card (see slide 34 for estimates of target steerage).

Low	High
10% below target	10% above target

Commercial Margin Assumptions

KBA observes a Franciscan Missionaries of Our Lady Health System 42% commercial margin. Conservatively, we model the DTE to be even with commercial rates given prices 10% below market, but with a 10 percentage-point increase through friendlier billing practices and improved patient collections with the patient card.

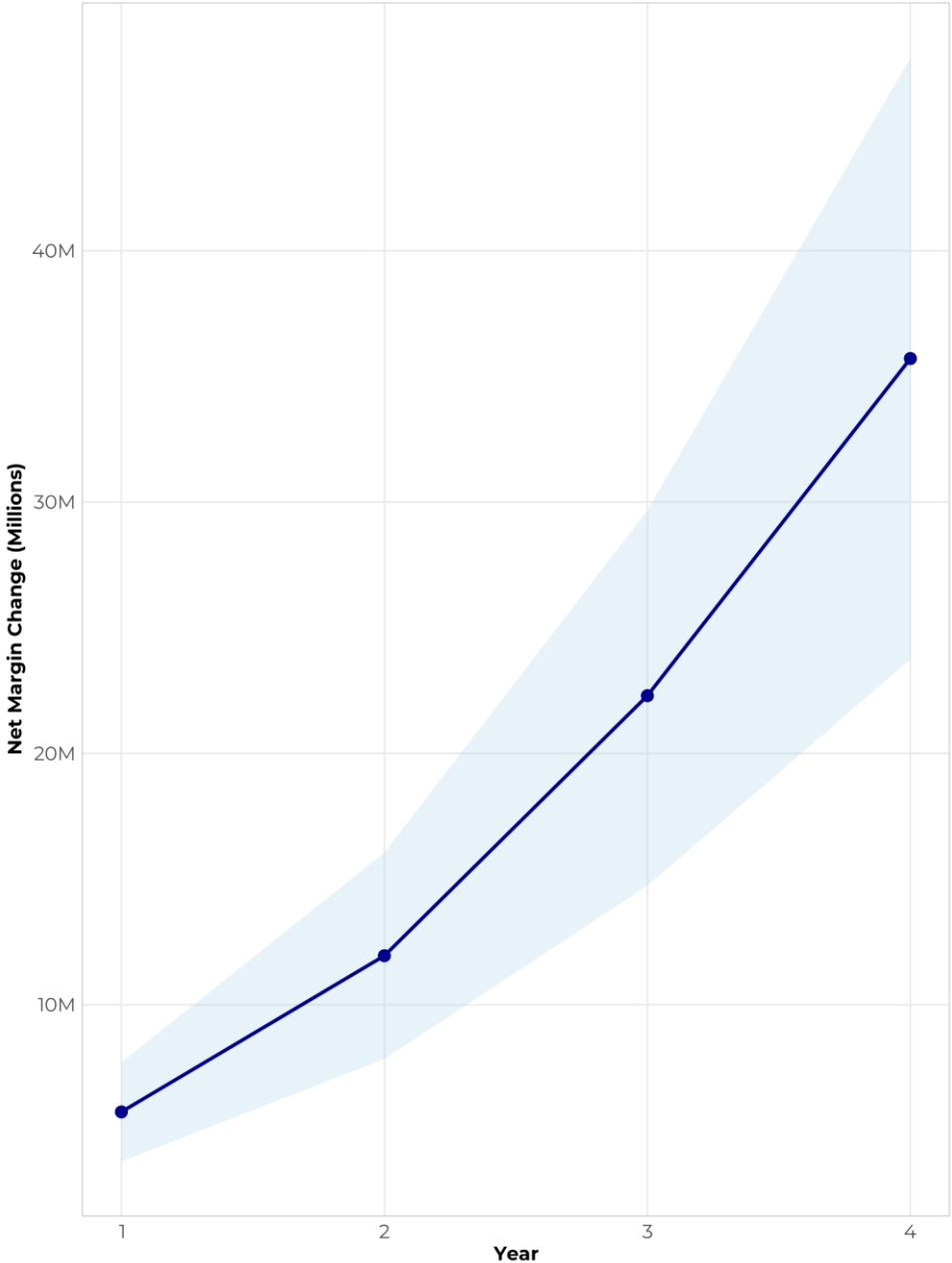
Low	Target	High
28.6%	33.6%	38.6%

FOUR-YEAR PROJECTIONS

Net Margin Change Over Years

Variation by Member Growth With Target Estimate Assuming

50%+ Within-Plan Steerage and a Commercial Margin of 33.6%



Target Member Growth				
Metric	Year 1	Year 2	Year 3	Year 4
Number of Members	3,700	7,400	12,200	18,300
Incremental Margin	6,023.3K	12,042.7K	22,331.4K	35,673.4K
Network Access Fee	44.4K	88.8K	146.4K	219.6K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	5,737.7K	11,951.5K	22,297.8K	35,713.0K

High Member Growth				
Metric	Year 1	Year 2	Year 3	Year 4
Number of Members	4,900	9,900	16,200	24,400
Incremental Margin	7,976.9K	16,111.2K	29,653.2K	47,564.6K
Network Access Fee	58.8K	118.8K	194.4K	292.8K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	7,705.7K	16,050.0K	29,667.6K	47,677.4K

Low Member Growth				
Metric	Year 1	Year 2	Year 3	Year 4
Number of Members	2,500	4,900	8,100	12,200
Incremental Margin	4,069.8K	7,974.2K	14,826.6K	23,782.3K
Network Access Fee	30.0K	58.8K	97.2K	146.4K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	3,769.8K	7,853.0K	14,743.8K	23,748.7K

DIRECT-TO-EMPLOYER STRATEGY:

ONLY AVAILABLE TO *PREMIER* HEALTH SYSTEMS



Franciscan
Missionaries
of Our Lady
HEALTH SYSTEM

STRONG Brand Quality

EXTENSIVE Patient Access

AFFORDABLE Value Pricing

EXCELLENT Care Quality

DTE Covered Members Financial Analysis

Target Estimate Assuming 50%+ Within-Plan Steerage and a Commercial Margin of 33.6%.

DTE Members	Pre-DTE Revenue	Pre-DTE Margin	DTE Revenue	DTE Incr. Revenue	DTE Total Margin	DTE Incr. Margin	Net Margin Less DTE Fees
100	\$ 177K	\$ 59K	\$ 658K	\$ 481K	\$ 221K	\$ 162K	\$ -167K
500	\$ 885K	\$ 297K	\$ 3,290K	\$ 2,405K	\$ 1,105K	\$ 808K	\$ 484K
1,000	\$ 1,770K	\$ 595K	\$ 6,580K	\$ 4,809K	\$ 2,211K	\$ 1,616K	\$ 1,298K
2,500	\$ 4,426K	\$ 1,487K	\$ 16,449K	\$ 12,023K	\$ 5,527K	\$ 4,040K	\$ 3,740K
10,000	\$ 17,704K	\$ 5,949K	\$ 65,797K	\$ 48,093K	\$ 22,108K	\$ 16,159K	\$ 15,949K
20,000	\$ 35,408K	\$ 11,897K	\$ 131,595K	\$ 96,186K	\$ 44,216K	\$ 32,319K	\$ 32,229K

- NOTE: Please see detailed financial analysis and assumptions used starting on page 23.
- Financial projections do not include 340B contribution margin, which could represent \$136 PMPM by optimizing 340B through Rx plan design, Franciscan Missionaries of Our Lady Health System's approved site(s) with network integration, and development of Franciscan Missionaries of Our Lady Health System's proprietary PBM program.

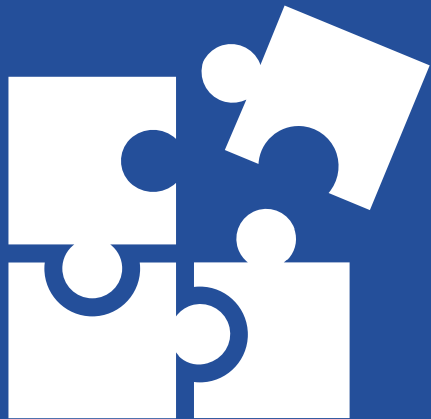
INTEGRATION WITH EPIC EHR SYSTEM

**KBA shares
evidence-based
standards of care for
27 chronic conditions
directly into Epic**



Hyperthyroidism: Yearly Clinical Evaluation, TSH and T4

THE MISSING PIECE TO VALUE- BASED CARE



- Patient engagement is the weak link in delivering value-based care; Franciscan Missionaries of Our Lady Health System's clinical leadership and employer participation can drive better outcomes.
- KBA's proactive care-management team supplements your care-management efforts in outreach to clients.
- Franciscan Missionaries of Our Lady Health System excels in Standards of Care completion rates for the Chronically Ill, improving care and costs.
- Integrating Franciscan Missionaries of Our Lady Health System's managed care with KBA's programs positions Franciscan Missionaries of Our Lady Health System as the value leader.

TRANSPARENT PBM INTEGRATION

- Support of Franciscan Missionaries of Our Lady Health System's retail pharmacy(s)
- Optimize Franciscan Missionaries of Our Lady Health System's 340B drug pricing program
- Allows Franciscan Missionaries of Our Lady Health System to create its own PBM program if desired
- Custom telephone number with personalized scripting and separate call tracking & reporting
- Fully customizable member welcome booklet & materials
- White labeled formulary document and marketing materials

CUSTOM CLIENT SUPPORT:

- Specialized call center capabilities
- Fully customizable plan designs
- Operational access to the platform
- Customized process and functions to promote DTE strategy

CLINICAL FLEXIBILITY:

- Full portfolio of custom formulary offerings – with optimized formulary shield
- Clinical programs that drive towards overall DTE strategy
- Options for additional drug savings by driving members to lower cost alternatives
- Flexibility to customize or layer on utilization management programs

STOP-LOSS INSURANCE

THE KBA EXPERIENCE

- Deep relationships with 20+ stop loss underwriters and carriers
- Over \$150M in annual stop loss premium placement
- Comprehensive pricing and medical service analysis performed for each DTE health plan to ensure proper pricing
- KBA provides the administration required of the stop-loss markets:
 - Bordereau Reporting
 - Claim Reporting and Reconciliation
 - Evaluation of Networks
 - Care Management
 - Initial Underwriting
 - Negotiation of rates and terms
 - Plan Design Actuarial

Tokio Marine

Nationwide

Berkshire

Hathaway

Sun Life

Liberty Mutual

Granular (Google)

Companion Life

Crum & Forster

American Fidelity

RGA

DTE PROGRAM MARKETING & SALES

KBA WORKS WITH YOU TO MANAGE & SELL:

- Branding & Messaging
- Direct & Brokerage Sales Distribution – Focused Sales Representative
- Brochures, Web Site, Media, etc.
- Stop-Loss/Level Funded Down to 20 EE Lives
- Sell Through Established Brokers in Your Service Area

COMMUNITY BASED SALES STRATEGY



Locally based sales rep with strong community ties



Collaboration with DTE provider partners to identify strategic employers for early adoption and sales



Work with select broker distribution partners with aligned incentives and goals



Pipeline reporting and collaboration with our DTE provider partners



Marketing and promotion of the new option in the community

COMPLETE MARKETING SYSTEM

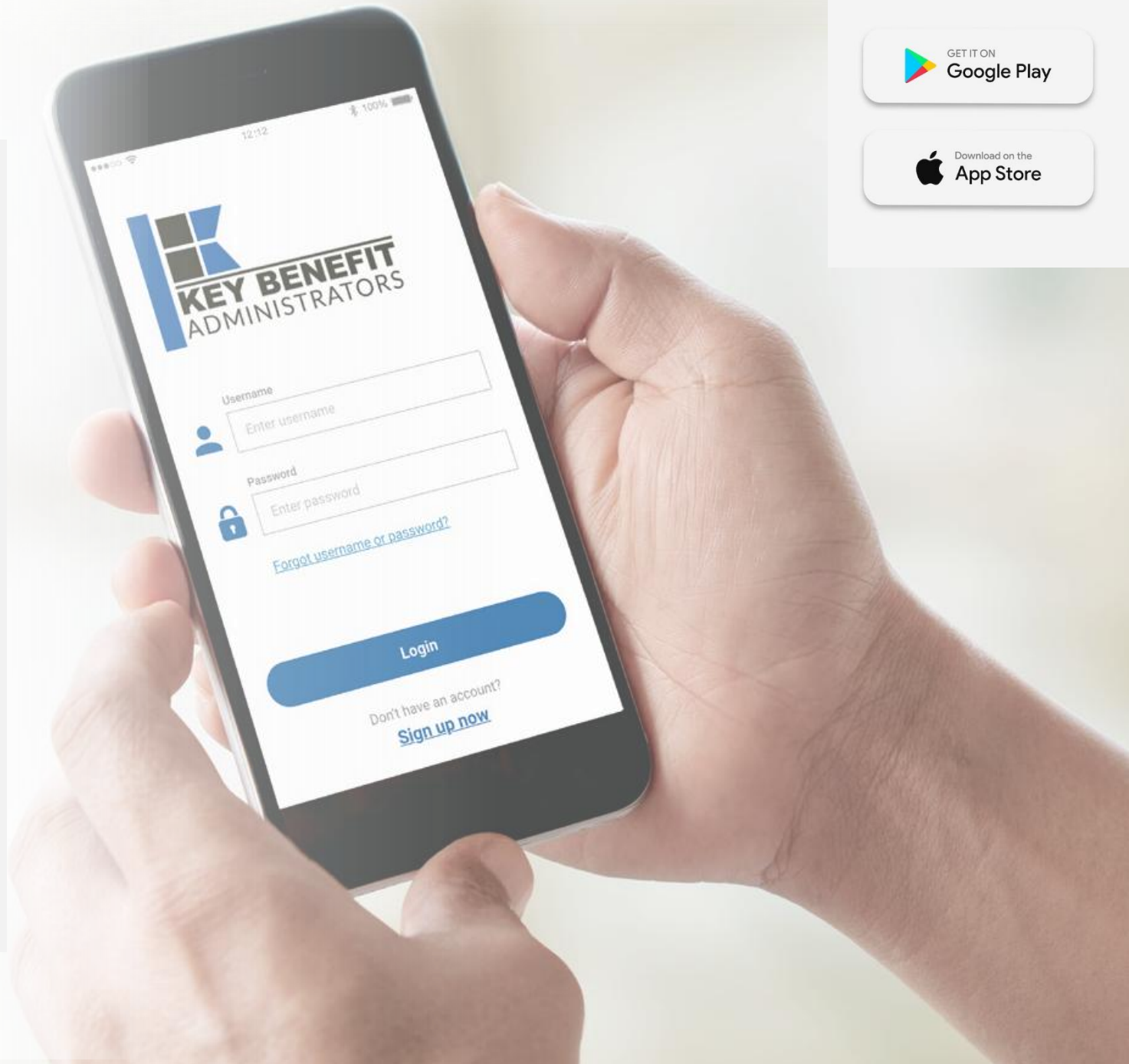
- Customized Branding and Messaging
- Customized Brochures
- Customized Web Site
- Targeted Employer Outreach
- Sales System (Social Media, Email, Text, etc.)



Web Portal and Mobile Application

Gives you accessibility to your benefit plan information, claims data, eligibility details and much more!

- Biometric Login for ease of access (Fingerprint & Facial Recognition)
- Simplified Accumulator View
- Recent Claims Summary and Full Detail
- Benefit Information
- View/Request an ID Card (Display - Print – Fax – Email)
- SSO directly to Vendor Solutions
- Rx Price Comparison and Pharmacy Discount Programs (if applicable)
- Express Requests to Customer Support
- Push Notifications (Mobile App)



GET IT ON
Google Play

Download on the
App Store

The Key Incentive Plan

Base Medical + Preferred Plus Benefits = Your Deductible Can Simply Vanish

Example: Member with a \$2,500 deductible / \$4,500 max out-of-pocket has a heart attack and cardiac surgery, resulting in a 5-day in-patient stay. In this example, the member would have a \$4,500 OOP with \$5,050 Preferred Plus Benefits for a **net OOP \$0 due to the preferred provider versus \$4,500 OOP exposure at any non-preferred provider.**

BASE MEDICAL BENEFITS*		PREFERRED PLUS BENEFITS		
Deductible		Benefits automatically included in your plan!		
Single	\$2,500		Preferred Provider	Non-Preferred Provider
Family	\$4,500	OP Radiology/X-Ray	\$200	\$0
Coinsurance		OP Diagnostic Labs	\$100	\$0
80% / 20%		Preferred Hospital Stay (\$500/day, 5 days)	\$2,500	\$0
Maximum Out-of-Pocket		Critical Illness Benefit	\$2,000	\$0
Single	\$4,500	Health Screening Benefit	\$125	\$0
Family	\$9,000	Total Employee OOP	\$4,500	\$4,500
Chronic Disease Management Regimens of Care Covered at 100%		Preferred Plus Benefits	\$4,925	\$0
Healthcare Convenience Card to Cover Out-of-Pocket Expenses at 0% interest		Employee Amount Due to Provider	\$0	\$4,500
Health Guardian Account to help members manage their healthcare expenses at Preferred Providers		Additional Benefit Payable to the Health Guardian Account	\$425	\$0

*Base Medical Benefits are for illustration purposes only. Base benefits can be customized to meet client needs.

THE BENEFIT STEERING THE PATIENT TO FRANCISCAN

What Will this Claim Cost Me?



Franciscan Missionaries of Our Lady Health System patients can have all out-of-pocket liability reduced to \$0!

KEY BENEFIT
ADMINISTRATORS

Explanation of Benefits	
Heart Attack, Cardiac Surgery	
THE CHARGES	
Medical Bills	\$25,000
Healthcare out-of-pocket due to provider	\$4,500
Since your out of pocket maximum has been met, the rest of your eligible charges this year will be paid at 100%	
THE CREDITS	
OP Radiology/X-Ray	\$200
OP Diagnostic Labs	\$100
Preferred Hospital Stay (\$500/day, 5 days)	\$2,500
Critical Illness Benefit	\$2,000
Health Screening Benefit	\$125
Benefit Total	\$4,925
Out of Pocket Paid to Hospital	(\$4,500)
Balance Paid to HGA	\$425

THE KBA HEALTHCARE CONVENIENCE CARD

Creating EPO Steerage in a PPO Network

The Healthcare Convenience Card is eligible to be used only for specific merchant codes.

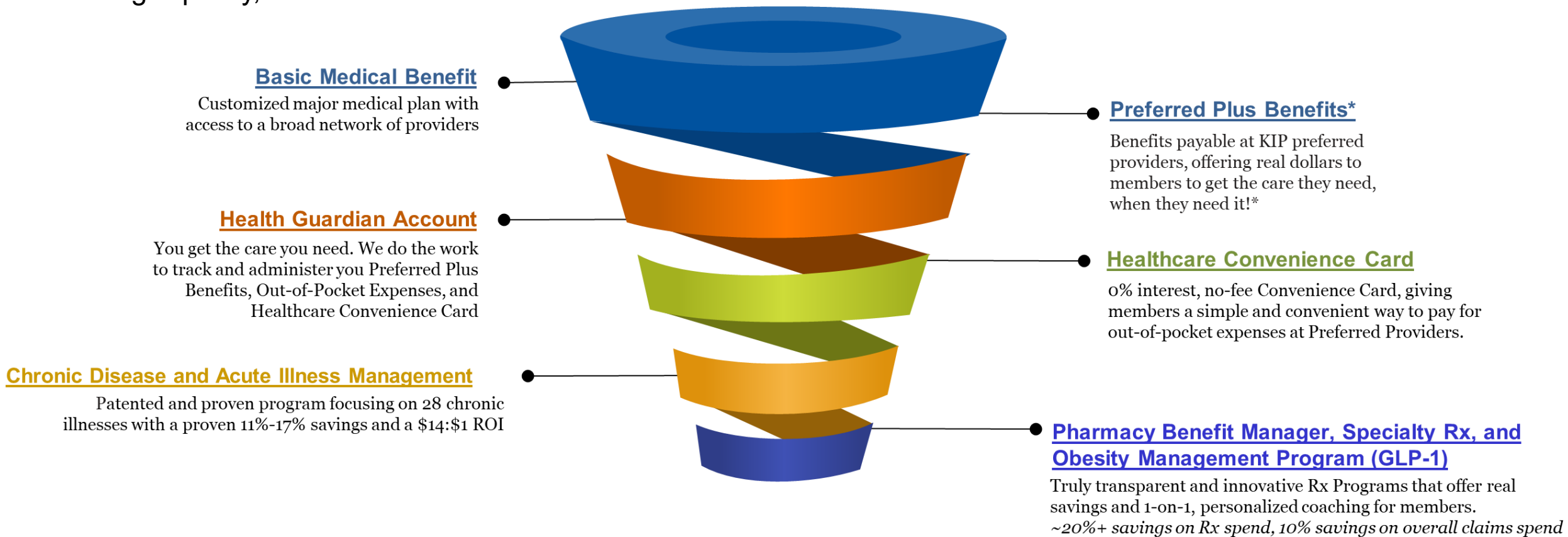
1. To utilize at Specific Healthcare Providers
 - a. Designated Hospitals
 - b. Urgent Care Facilities
 - c. Physicians
 - d. Eligible Ancillary Providers
2. Out of Pocket Expenses Qualify
3. 0% Interest Rate
4. Employee Determines Repayment Schedules
5. To Qualify: Answer One Simple Question



The Key Incentive Plan

Affordability Without Compromise

The **Key Incentive Plan** is a one-of-a-kind solution, empowering employers to take back control of their healthcare programs, that offers freedom of choice when choosing providers in your local community, while incenting members to choose high-quality, cost-effective care.



Weight Management (GLP-1s) that Don't Break the Bank

The GLP-1 medication market has experienced explosive growth, fundamentally reshaping healthcare spending across America. Prime Therapeutics reports that per-member-per-month spend has tripled by March 2025, with these breakthrough medications now representing approximately 15% of total pharmacy spend. More than 1 in 50 members filed a GLP-1 claim in Q1 2025 alone, demonstrating unprecedented demand for these life-changing treatments.

The Challenge

Traditional GLP-1 medications cost between \$1,089-\$1,349 monthly, creating unsustainable budget pressures for employers while limiting employee access to essential treatments.

Our Solution

Key Benefits secures generic GLP-1 medications for just \$249 per month, enabling comprehensive workforce health while maintaining strict budget control and financial stability.

The Impact

Deliver a healthier population, increased employee satisfaction, and sustainable costs that support long-term organizational success and employee wellbeing.

Proven Results: \$7M Annual Savings

One major city spending \$11 million annually on GLP-1 medications achieved \$7 million in cost savings through our comprehensive approach. This dramatic reduction demonstrates how strategic sourcing combined with clinical support creates sustainable, scalable solutions for large employers facing escalating pharmaceutical costs.

Beyond Medication: Comprehensive Care

Our program extends far beyond cost-effective medication access. Members receive specialized coaching twice monthly, focusing on maintaining muscular strength, engaging in moderate exercise, and developing sustainable lifestyle changes that support long-term weight maintenance after GLP-1 treatment completion.

270

Health Conditions

Created or exacerbated by obesity according to Cleveland Clinic research

\$8.5K

Additional Spend

Average annual healthcare costs above normal for obese individuals (KFF)

✔ **Transform Your Healthcare Strategy:** Join forward-thinking employers who are providing comprehensive weight management solutions while protecting their bottom line. Our approach combines clinical excellence with fiscal responsibility, creating a win-win for organizations and their valued employees.

The Healthcare Provider Landscape Challenge

The Louisiana/Mississippi healthcare market presents a complex web of care providers, specialty groups, and healthcare facilities that can overwhelm patients, employers, and payers alike.

Market Complexity

- Hundreds of independent provider groups
- Multiple health system affiliations
- Varying quality and pricing structures
- Limited transparency in outcomes

The Challenge

Without strategic curation and management, this overwhelming number of care groups creates confusion, inefficiencies, and suboptimal outcomes for all stakeholders.

The solution: A carefully managed, high-value network.



Our Goal in Partnering With FMOLHS



Build the Highest-Value Network

Create Louisiana's premier healthcare network that delivers exceptional quality care with market-leading pricing, setting the gold standard for value-based healthcare delivery.



Enable Fastest-Possible Growth

Leverage our proven methodologies and established FMOLHS relationships to accelerate network expansion, ensuring rapid market penetration and comprehensive coverage across the region.



Vault FMOLHS to Competitive Advantage

Position FMOLHS as the anchor and leader of this transformative network, creating sustainable competitive advantages that drive market leadership and long-term success.

"Value is high-quality care at a great price – this principle guides every decision we make in network development and provider selection."



HEALTHCARE
PARTNERSHIP

Client Service Team Development Process

Our approach to building your dedicated KBA service team is a structured, collaborative process designed to ensure optimal support and alignment with your specific needs.



Introduction & Acquaintance

- On-site client visit by VP, Client Relations.
- Education on HR Team roles, goals, and objectives.
- Collaboration on HR team's wants, needs (reporting, data, support).
- Identify frequency and format for in-depth plan performance reviews and analytics.



Ongoing Team Growth

- As consulting, data analytics, and day-to-day service progress, additional resources will be assessed and introduced to clients as needed to ensure continuous improvement and support.



Tailored KBA Service Team

- Service team customized and assigned based on client wants, needs, expectations, and initial assessment.
- Identifying key characteristics of the client's leadership and HR team in conjunction with the evaluation of the KBA team to apply the appropriate level of expertise and support.



KBA DEDICATED TEAM ROLES

Our expert team is committed to delivering tailored solutions and exceptional service, ensuring consistent support and strategic guidance for every client.

Client Vice President (CVP)

Average Tenure: 15 Years

A dedicated individual delivering solutions tailored to client needs, driven by their unique data. The KBA CVP is deeply knowledgeable about client demographics, goals, and plans, as well as the health status of their membership. Through unparalleled analytics, the CVP illuminates the intelligence of the data, coordinates and conducts plan performance reviews, and provides meaningful recommendations. The CVP can also assist with employee enrollment meetings, presentations, and announcement materials, staying informed on client issues and initiatives, and tracking all account-related information.

Benefit Consultant (BC)

Average Tenure: 5 Years

Delivers strategic consultation to the client in conjunction with the CVP, with a primary focus on reporting and strategic insights. During renewal season, the BC assists in preparing comprehensive renewal proposals that include detailed service offerings, clear fee structures, and defined timelines. They also coordinate and conduct employee enrollment meetings as needed, ensuring a seamless and informed process for all stakeholders.

Account Representative (AR)

Average Tenure: 3 Years

Manages the day-to-day needs of the account, ensuring consistent communications, timely follow-up services, and continuous account monitoring. The AR oversees new case implementation and renewal processes alongside the KBA operations team. Responsibilities include the distribution of monthly reports, facilitating plan design changes, and obtaining signatures for plan documents and amendments. Additionally, they assist with funding management, including ACH transfer of funds and various other inquiries to ensure smooth financial operations.



ADDITIONAL KBA TEAM MEMBERS

On-Site KBA Support

An on-site resource for members to ask questions regarding benefits, eligibility, or claims. This role also provides essential HR support for plan administration, ensuring smooth operations and direct assistance.

Economist/Analyst

George Avery, PhD, MPA: Dr. Avery holds a doctorate in Health Services Research, Policy, and Administration from the University of Minnesota. With extensive academic experience as a professor at Purdue University and the University of Minnesota Duluth, he has contributed significantly to his field. His impressive publication record includes six book chapters and over 20 peer-reviewed journal articles.

Dr. Avery's expertise is highly sought after, evidenced by his advisory roles for prestigious organizations such as the Indiana Office of Rural Health, the Centers for Disease Control and Prevention, the Department of Defense, and the United States Environmental Protection Agency. He brings over 30 years of invaluable experience in public health, health economics, and econometric analysis to the team.



Analytics Engine - EZViewSM

Turning Data into Actionable Intelligence

The screenshot displays the EZView Analytics Engine web application. At the top, a navigation bar contains tabs for 'Who, When, Where, How?', 'What?', 'Networks', 'Claimant Demographics', 'Claim Costs Summary', 'CDM - Chronic Conditions', 'CDM - Member Detail', and 'Claimants'. The 'Who, When, Where, How?' tab is currently selected. Below the navigation bar, the EZView logo is prominently displayed on the left, and the American Health Data Institute logo is on the right. The main content area features four large, dark gray rectangular cards arranged horizontally. Each card contains a white icon, a question, and a brief description of the analysis function. The first card shows an icon of three people and asks 'Who?', with the description 'Select Population for Analysis Based on Data Source, Group, Plan, or Location'. The second card shows a calendar icon with the number 31 and asks 'When?', with the description 'Select Incurred, Paid and/or Eligibility Time Periods of Analysis'. The third card shows a globe icon and asks 'Where?', with the description 'Select Population for Analysis Based on Geography'. The fourth card shows a question mark icon and asks 'How?', with the description 'Open the Users' Manual'. At the bottom center of the interface, the version number 'v. 2.15' is displayed.

Who, When, Where, How? What? Networks Claimant Demographics Claim Costs Summary CDM - Chronic Conditions CDM - Member Detail Claimants

EZView American Health Data Institute

Who?
Select Population for Analysis
Based on Data Source, Group,
Plan, or Location

When?
Select Incurred, Paid and/or
Eligibility Time Periods of
Analysis

Where?
Select Population for Analysis
Based on Geography

How?
Open the Users' Manual

v. 2.15



Population Distribution

HEZ	Members with Conditions	% Of Members	Average Modifiable Risk Score
Total/Average	42,626	2.6%	8.2
ABBEVILLE LA	1	2.5%	4.9
ABBEVILLE SC	1	0.9%	16.0
ABERDEEN SD	1	1.4%	4.3
ABERDEEN WA	1	0.1%	11.4

Members and Cost

MRS Stratum	Members	Paid	Average Cost
Total/Avg	42,626	\$907,466,521	\$21,289
1	21,171	\$267,124,512	\$12,617
2	14,767	\$337,715,521	\$22,870
3	4,475	\$151,167,994	\$33,781
4	2,213	\$151,458,494	\$68,440

Condition Distribution

Chronic Illness	Members with Condition(s)	% Of Members
Total Members	42,626	2.6%
Hypertension	19,957	1.2%
Hyperlipidemia	18,225	1.1%
Depression	10,328	0.6%
Asthma	7,245	0.4%
Diabetes Mellitus	7,059	0.4%
Hypothyroidism	5,727	0.3%
Sleep Apnea	5,134	0.3%
Pre-Diabetes	2,994	0.2%
Coronary Artery Disease	2,590	0.2%
Chronic Obstructive Pulmonary Disease	2,028	0.1%
Chronic Kidney Disease	1,187	0.1%
Atrial Fibrillation	1,028	0.1%
Rheumatoid Arthritis	763	0.0%
Connective Heart Failure	691	0.0%

Minimum Care Rates

Condition	MCR
Average Minimum Care Rate	35.2%
Parkinson's Disease	67.9%
Multiple Sclerosis	65.9%
Regional Enteritis	55.2%
Atrial Fibrillation	54.7%
Rheumatoid Arthritis	49.7%
Hyperlipidemia	48.1%
Hypertension	42.8%
Ulcerative Colitis	41.2%
Epilepsy	40.9%
Hypothyroidism	39.9%
Sleep Apnea	37.1%
Depression	34.7%
Coronary Artery Disease	30.6%
Diabetes Mellitus	25.3%
Concognitive Heart Failure	24.0%
Hyperthyroidism	24.0%

Service Rates

CDM Required Service	Required	Svc. Rate
Total/Average	190,563	45.3%
Urine Albumin/Creatinine	6,484	57.5%
Serum Calcium	1,187	56.1%
Serum Potassium	1,878	55.7%
Serum Creatinine	28,894	54.1%
TSH	6,069	53.2%
T-Cell/CD-4 Count	96	52.1%
CBC	272	49.6%
Hgb or Hct	1,187	48.6%
Glycohemoglobin	14,118	47.6%
Lipid Panel	28,801	47.3%
LDL	2,849	47.0%
HIV Quantification	192	46.4%
E & M Visit	84,647	43.8%
FBS or HgbA1c	3,517	34.3%
T4	342	33.6%

Current Selections

12

?

Additional Selections

Illness Identified > 12 Months	☐
CDM Activity Status	☐
Member Compliance Status	☐
Condition Compliance Status	☐
Service Compliance Status	☐

All Patient, Provider, and Location Data Regardless of Network Affiliation

Identify the sickest of the sick

Identify Every Chronic Condition by Patient for Intervention

Monitor Compliance with Nurse Coaching and Triage to Providers

Regimens of Care for Chronic Illnesses

Key Capabilities of EZViewSM Analytics Engine



Comprehensive Data Integration

Access all patient, provider, and location data, regardless of network affiliation, for a unified view of healthcare operations.



High-Risk Patient Identification

Proactively identify the "sickest of the sick" to enable timely interventions and improve patient outcomes.



Chronic Condition Mapping

Accurately identify every chronic condition by patient, allowing for targeted intervention strategies.



Compliance & Triage Monitoring

Monitor compliance with nurse coaching protocols and efficiently triage patients to appropriate providers based on their needs.



Standardized Care Regimens

Implement and track standardized regimens of care for chronic illnesses, ensuring consistent, high-quality treatment.



Measuring Success: Customer Experience

Key Performance Indicators

Metric	2025 YTD	Industry Benchmark	How We Compare	✓ Industry-leading responsiveness 1.04% abandonment and 17 s average speed of answer—minimal dropped calls, maximum access. ✓ Outstanding quality & access 90% of calls answered within 30s (vs. 80% target) and 99%+ audit scores across voice, email & chat. ✓ Balanced efficiency & depth Handle time reflects the detailed, compassionate TPA support your members need.
Abandonment Rate	1.04%	3–5% (acceptable), <2% (best-in-class)	Excellent 🍌	
Average Speed of Answer (ASA)	17 seconds	<28 seconds (typical target); ~3 min in healthcare BPO	Excellent 🍌	
SLA (80% ≤ 30 sec)	90.00%	80% in 20–30 seconds (standard)	Above target ✓	
Average Handle Time	7 minutes 47 seconds	4–6 minutes typical; 7–8+ mins for complex TPA calls	Higher than industry, but expected	
After Call Work (ACW)	49 seconds	30–60 seconds typical	Within range 🟢	
Quality Audits (Email, Chat, Soft Skills, Procedural)	99%+	95–97% considered strong	Outstanding 100	

KBA 2023-2024 Total Quality Management Statistics

300+
TOTAL QUALITY MEASURES

50,000+
UNIQUE AUDITS ANNUALLY

\$200M+
PREMIUM & CLAIMS REVIEWED ANNUALLY

New Business / Renewals				
New Case Accuracy	2023 KBA 99.86%	2024 KBA 100.00% ↑	INDUSTRY N/A	GOAL 99.75%
Renewal Case Accuracy	2023 KBA 99.96%	2024 KBA 100.00% ↑	INDUSTRY N/A	GOAL 99.75%
Fulfillment Materials Audit	2023 KBA 99.99%	2024 KBA 99.98% ↓	INDUSTRY N/A	GOAL 100%

Enrollment Center				
Soft Skills Audit	2023 KBA 99.43%	2024 KBA 99.22% ↓	INDUSTRY N/A	GOAL 98.00%
Procedural Skills Audit	2023 KBA 98.26%	2024 KBA 98.23% ↓	INDUSTRY N/A	GOAL 97.00%

Billing & Eligibility Quality				
Eligibility Accuracy	2023 KBA 100.00%	2024 KBA 100.00%	INDUSTRY N/A	GOAL 99.75%
Plans/Rates/Coverage Audits	2023 KBA 100.00%	2024 KBA 100.00%	INDUSTRY N/A	GOAL 99.75%
Cash Receipts Logging	2023 KBA 99.97%	2024 KBA 99.78% ↓	INDUSTRY N/A	GOAL 98.00%
Cash Posting Accuracy	2023 KBA 99.96%	2024 KBA 99.78% ↓	INDUSTRY N/A	GOAL 97.00%

Claims Quality				
Turn Around Time (Days)	2023 KBA 3.79	2024 KBA 6 ↓	INDUSTRY 10	GOAL 7
% Paid within 10 Bus Days	2023 KBA 88.90%	2024 KBA 87.34% ↓	INDUSTRY 80.00%	GOAL 85.00%
Financial Accuracy	2023 KBA 99.30%	2024 KBA 99.44% ↑	INDUSTRY 99.00%	GOAL 99.00%
Procedural Accuracy	2023 KBA 99.00%	2024 KBA 99.16% ↑	INDUSTRY N/A	GOAL 95.00%
% of Claims Audited	2023 KBA 3.77%	2024 KBA 3.80% ↑	INDUSTRY 1%	GOAL 3%

Customer Experience Quality				
Average Speed to Answer	2023 KBA 22 sec	2024 KBA 44 sec ↓	INDUSTRY <45s	GOAL <30s
Soft Skills Audit	2023 KBA 99.50%	2024 KBA 99.54% ↑	INDUSTRY N/A	GOAL 98.00%
Email Correspondence Audit	2023 KBA 100.00%	2024 KBA 100.00%	INDUSTRY N/A	GOAL 98.00%
Abandonment Rate	2023 KBA 1.00%	2024 KBA 1.78% ↓	INDUSTRY <5%	GOAL <1%
% Answered w/in 45 Sec	2023 KBA 84.90%	2024 KBA 75.00% ↓	INDUSTRY 80-60%	GOAL 80-30%
Procedural Audit	2023 KBA 99.22%	2024 KBA 99.10% ↓	INDUSTRY N/A	GOAL 97.00%
Web Chat Corresp. Audit	2023 KBA 99.64%	2024 KBA 99.50% ↓	INDUSTRY N/A	GOAL 98.00%

The image is a composite. The upper portion shows a panoramic view of a city skyline, likely New York City, with the Empire State Building as a prominent feature, seen through a window with vertical frame lines. The lower portion shows a close-up of a desk with several sheets of paper containing financial charts and tables. A black pen, a white coffee cup on a saucer, and a calculator are also on the desk. A semi-transparent white banner is placed across the middle of the image, containing the title text.

FINANCIAL PROJECTIONS & ASSUMPTIONS

ESTIMATION OF FMOLHS'S 2022 PAYER MIX

Payer Group	Total Inpatient Days	Revenue	Payer Mix
All Franciscan Missionaries of Our Lady Health System	463,500	\$2,951,800K	100%
Medicare	109,013	\$439,048K	22%
Medicaid	80,505	\$742,918K	16%
Other Government Pay	0		
No-Pay, Charity Care	13,965	-\$91,207K	3%
Commercial/Self Pay	260,017	\$1,770,336K	59%

Values in blue are taken from the 2022 Medicare Provider Cost Report (<https://data.cms.gov/provider-compliance/cost-report/hospital-provider-cost-report/data>)

Values in yellow are taken from Definitive Healthcare data.

Total Medicare patient revenue is reported by CMS in the Medicare Provider Analysis and Review Report (MedPAR)

Estimating Payer Mix

- Medicare, Medicaid, and other government payer mixes are estimated as the percentage of inpatient days for each group divided by total inpatient days.
- No-pay and charity care is estimated as the percentage of no-pay/charity care compared to total overall revenue.
- Commercial/self pay is assumed to be the rest of the total payer mix share.

Estimating Total Inpatient Days

- Total inpatient days for 'No Pay' and 'Commercial' are estimated to be the payer mix estimates for "No Pay, Charity Care" and "Commercial/Self Pay" multiplied by the total number of inpatient days.

In other analysis, the outpatient payer mix is assumed to mirror the inpatient payer mix.

ESTIMATION OF MARGIN FOR COMMERCIAL VS. GOVERNMENT PAY

The RAND data for Franciscan Missionaries of Our Lady Health System estimates commercial insurance reimbursements at 268% of Medicare Rates

Estimating Commercial Margin

(Calculated through counterfactual of assuming all CMS/Government pay was paying at commercial rates)

Commercial Revenue	\$1,770,336K
Gov-pay revenue at 268% current rate	\$3,169,065K
Total Revenue	\$4,939,401K
Total Costs	\$2,876,237K
Net Income	\$2,063,164K
	42%

Estimating Gov-Pay Margin

(Calculated through counterfactual of assuming all commercial pay was paying at Medicare rate)

Commercial Revenue	\$660,282K
Divided by 268%	
Gov-pay revenue	\$1,181,966K
Total Revenue	\$1,842,248K
Total Costs	\$2,876,237K
Net Income	\$ -1,033,989K
	-56%

ESTIMATING MARKET SHARE

Hospital Name	Number of Beds	Inpatient Total Charges	Outpatient Total Charges	Inpatient Percent of Market Total	Outpatient Percent of Market Total (Assuming 49.1% of Physicians Employed by Hospitals)
Our Lady of the Lake Regional Medical Center	647	\$2,105,119.7K	\$2,245,033.2K	10%	4%
Our Lady of Lourdes Regional Medical Center	383	\$1,109,867.0K	\$ 969,535.7K	5%	2%
St Francis Medical Center	332	\$ 816,545.2K	\$ 787,455.0K	4%	2%
Our Lady of the Lake Surgical Hospital	37	\$ 68,217.8K	\$ 142,022.4K	<1%	<1%
Our Lady of the Angels Hospital	36	\$ 45,007.9K	\$ 105,043.3K	<1%	<1%

25%
Inpatient
Market
Share

10%
Outpatient
Market
Share

For outpatient market share, it is estimated that only 49.1% of local physicians are employed by area hospitals (Research from the Physician Advocacy Institute:
<https://www.physiciansadvocacyinstitute.org/Portals/0/assets/docs/PAI-Research/PAI-Avalere%20Physician%20Employment%20Trends%20Study%202019-2023%20Final.pdf?ver=uGHF46u1GSeZgYXMKFyYvw%3d%3d>)

Hospital Metrics					
Metric	Our Lady of the Lake Regional Medical Center	Our Lady of Lourdes Regional Medical Center	St Francis Medical Center	Our Lady of the Angels Hospital	St Dominic Hospital
Total Inpatient Patient Revenue	\$701,005K	\$294,115K	\$194,338K	\$10,577K	\$282,902K
Total Outpatient Patient Revenue	\$747,596K	\$256,927K	\$187,414K	\$24,685K	\$252,241K
Total Medicare Revenue	\$143,420K	\$99,372K	\$75,131K	\$5,600K	\$115,525K
Estimated Medicaid and Other Government-Pay Revenue	\$491,520K	\$114,081K	\$56,708K	\$26,181K	\$54,427K
Estimated Commercial Revenue	\$814,002K	\$337,457K	\$250,254K	\$3,496K	\$365,128K
Total Uncompensated Care	\$48,085K	\$10,719K	\$8,514K	\$2,752K	\$21,138K
Estimated Inpatient Costs	\$695,301K	\$252,039K	\$195,221K	\$23,780K	\$266,674K
Estimated Outpatient Costs	\$741,513K	\$220,171K	\$188,266K	\$55,501K	\$237,771K
Total Operating Expenses	\$1,436,815K	\$472,210K	\$383,487K	\$79,281K	\$504,445K
Net Income	\$12,127K	\$78,700K	\$-1,394K	\$-44,004K	\$30,635K
Estimated Payer Mix Percent Medicare	16.1%	21.3%	25.1%	22.9%	35.6%
Estimated Payer Mix Percent Medicaid	3.0%	35.5%	29.5%	39.1%	17.3%
Estimated Payer Mix Percent Other Government-Pay	0.0%	0.0%	0.0%	0.0%	0.0%
Payer Mix Percent No Pay	3.2%	1.9%	2.2%	7.2%	3.8%
Estimated Payer Mix Percent Commercial	77.7%	41.3%	43.3%	30.8%	43.3%
Estimated Commercial Profit Margin	42.9%	48.1%	36.5%	10.6%	38.5%

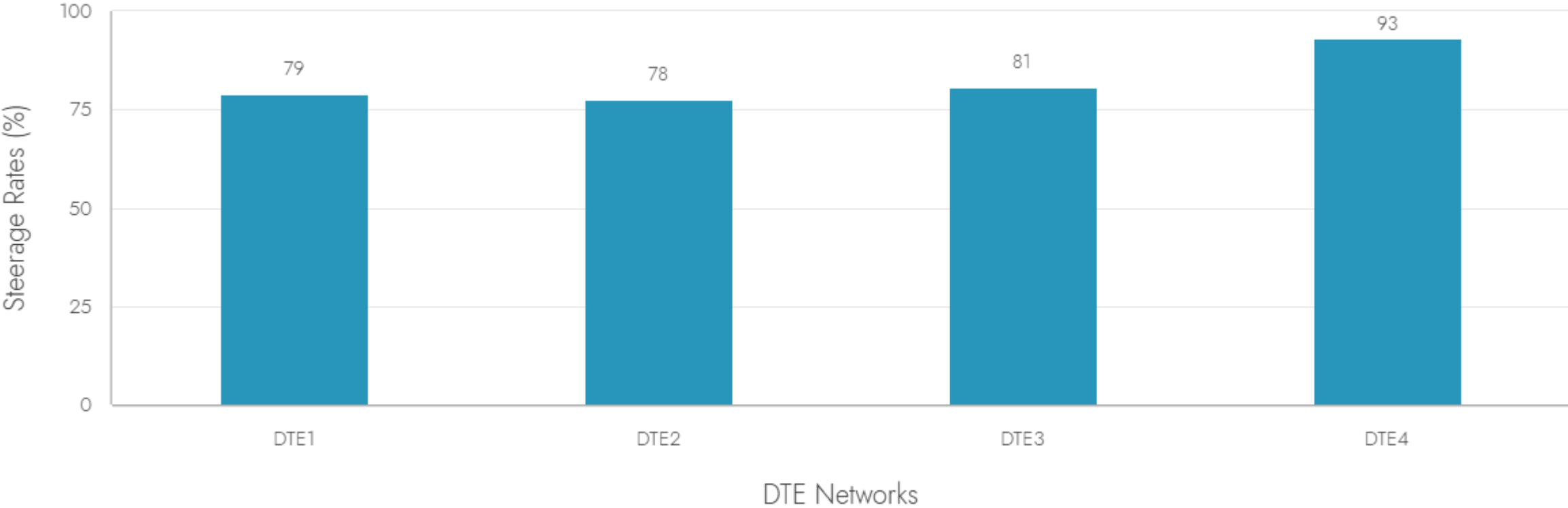
1. Most data is taken from the CMS provider cost report

2. Medicare costs are taken from the Medicare Provider Analysis and Review (MedPAR)

3. Data with 'Estimated' in the title are derived from other data points.

4. Market Share estimates based on total patient revenues with area hospitals. Ambulatory market share based on regional estimates of the percentage of local physicians employed by area hospitals. See Physician Advocacy Institute

STEERAGE RATES FOR CURRENT DTE NETWORKS



The average Tier 1 Steerage is 83%.

PROJECTED PLAN GROWTH

Year	Employees	Members	Percent of Members Choosing Franciscan Missionaries of Our Lady Health System for Inpatient Care	Percent of Members Choosing Franciscan Missionaries of Our Lady Health System for Outpatient Care
1	2,100	3,800	60%	50%
2	4,100	7,400	60%	50%
3	6,800	12,200	65%	55%
4	10,200	18,400	67%	58%
5	14,500	26,100	68%	60%
6	19,800	35,600	68%	60%
7	26,200	47,200	69%	61%
8	33,700	60,700	69%	62%
9	42,200	76,000	69%	62%
10	51,300	92,300	70%	63%
11	60,700	109,300	70%	63%
12	69,900	125,800	70%	64%
13	78,500	141,300	71%	64%
14	86,100	155,000	71%	65%
15	92,600	166,700	71%	66%
16	97,900	176,200	72%	66%
17	102,100	183,800	72%	66%
18	105,400	189,700	72%	67%
19	107,800	194,000	72%	67%
20	109,700	197,500	72%	67%

KBA currently estimates that Franciscan Missionaries of Our Lady Health System has 25% inpatient market share, and 10% outpatient market compared to hospitals/clinics in the Franciscan Missionaries of Our Lady Health System service area (see slide 8).

Based on previous DTE experience, KBA expects a significant bump in patient steerage to Franciscan Missionaries of Our Lady Health System.

Additional steerage will occur from additional steerage patient benefits and the patient card.

High estimates start nearly 10% higher than target with low estimates at 10% lower than target

YEAR 4 PROJECTED REVENUE AND PROFIT

Pre-DTE Cohort of 18,300 Employees	Inpatient	Outpatient	Professional	Total
Revenue†	\$ 15,728,866	\$ 11,363,151	\$ 5,306,495	\$ 32,398,512
Net Income‡	\$ 5,284,899	\$ 3,818,019	\$ 1,782,982	\$ 10,885,900

With DTE	Inpatient	Outpatient	Professional	Total
Revenue°	\$ 41,783,922	\$ 65,530,002	\$ 30,601,954	\$ 137,915,878
Net Income*	\$ 14,039,398	\$ 22,018,081	\$ 10,282,257	\$ 46,339,735

† Assuming proportional revenues from market shares of 25% (inpatient) and 10% (outpatient)

‡ Conservatively assuming a commercial margin of 33.6% (80% of the estimated commercial margin)

° Assuming 67% inpatient and 58% market steerage based on KBA narrow network DTE experience (see previous slide)

* Conservatively assuming a commercial margin of 33.6%, with a 10 percentage-point reduction to reflect competitive-edge rates and a 10 percentage-point increase due to lower billing costs and higher patient responsibility collections because of the member convenience card.

Total Increase in Net Income	\$ 35,453,835
Network Access Fee (Revenue) of \$1 PMPM	\$ 219,600
Total Profit Growth	\$ 35,672,835
Minus Implementation Fee (One-Time)	-\$ 0
Minus Sales and Marketing (Annual – Ongoing)	-\$ 180,000
Total	\$ 35,492,835

Target Estimate Assuming 50%+ Within-Plan Steerage and a Commercial Margin of 33.6%				
All values in thousands of dollars. Modeled With Year 1 Fees and Patient Steerage.				
Metric	500 DTE Covered Lives	1000 DTE Covered Lives	5000 DTE Covered Lives	25000 DTE Covered Lives
Pre-DTE Group Inpatient Revenue	\$429.8	\$859.5	\$4,297.5	\$21,487.5
Pre-DTE Group Inpatient Margin	\$144.4	\$288.8	\$1,444.0	\$7,219.8
DTE Group Inpatient Revenue	\$1,020.9	\$2,041.8	\$10,209.0	\$51,044.9
DTE Group Inpatient Margin	\$343.0	\$686.0	\$3,430.2	\$17,151.1
DTE Group Incremental Inpatient Revenue	\$591.1	\$1,182.3	\$5,911.5	\$29,557.4
DTE Group Incremental Inpatient Margin	\$198.6	\$397.3	\$1,986.3	\$9,931.3
Pre-DTE Group Outpatient Revenue	\$310.5	\$620.9	\$3,104.7	\$15,523.4
Pre-DTE Group Outpatient Margin	\$104.3	\$208.6	\$1,043.2	\$5,215.9
DTE Group Outpatient Revenue	\$1,546.7	\$3,093.4	\$15,466.8	\$77,334.0
DTE Group Outpatient Margin	\$519.7	\$1,039.4	\$5,196.8	\$25,984.2
DTE Group Incremental Outpatient Revenue	\$1,236.2	\$2,472.4	\$12,362.1	\$61,810.6
DTE Group Incremental Outpatient Margin	\$415.4	\$830.7	\$4,153.7	\$20,768.4
Pre-DTE Group Professional Revenue	\$145.0	\$290.0	\$1,449.9	\$7,249.3
Pre-DTE Group Professional Margin	\$48.7	\$97.4	\$487.2	\$2,435.8
DTE Group Professional Revenue	\$722.3	\$1,444.6	\$7,222.9	\$36,114.3
DTE Group Professional Margin	\$242.7	\$485.4	\$2,426.9	\$12,134.4
DTE Group Incremental Professional Revenue	\$577.3	\$1,154.6	\$5,773.0	\$28,865.0
DTE Group Incremental Professional Margin	\$194.0	\$387.9	\$1,939.7	\$9,698.6
Net Margin Less DTE Fees	\$484.0	\$1,297.9	\$7,809.7	\$40,368.3

Target Estimate Assuming 50%+ Within-Plan Steerage and a Commercial Margin of 33.6%

All values in thousands of dollars. Modeled With Year 1 Fees and Patient Steerage.

Metric	500 DTE Covered Lives	1000 DTE Covered Lives	5000 DTE Covered Lives	25000 DTE Covered Lives
Pre-DTE Group Total Revenue	\$885.2	\$1,770.4	\$8,852.1	\$44,260.3
Pre-DTE Group Total Margin	\$297.4	\$594.9	\$2,974.3	\$14,871.4
DTE Group Total Revenue	\$3,289.9	\$6,579.7	\$32,898.7	\$164,493.3
DTE Group Incremental Total Revenue	\$2,404.7	\$4,809.3	\$24,046.6	\$120,233.0
Incremental Total Revenue by Member	\$4.8	\$4.8	\$4.8	\$4.8
DTE Group Total Margin	\$1,105.4	\$2,210.8	\$11,053.9	\$55,269.7
DTE Group Incremental Total Margin	\$808.0	\$1,615.9	\$8,079.7	\$40,398.3
Wellness Share	\$6.0	\$12.0	\$60.0	\$300.0
Implementation Fee (One-Time)	\$150.0	\$150.0	\$150.0	\$150.0
Sales and Marketing Annual Fee	\$180.0	\$180.0	\$180.0	\$180.0
Net Margin Less DTE Fees	\$484.0	\$1,297.9	\$7,809.7	\$40,368.3

High Estimate Assuming 60%+ Within-Plan Steerage and a Commercial Margin of 38.6%				
All values in thousands of dollars. Modeled With Year 1 Fees and Patient Steerage.				
Metric	500 DTE Covered Lives	1000 DTE Covered Lives	5000 DTE Covered Lives	25000 DTE Covered Lives
Pre-DTE Group Total Revenue	\$885.2	\$1,770.4	\$8,852.1	\$44,260.3
Pre-DTE Group Total Margin	\$297.4	\$594.9	\$2,974.3	\$14,871.4
DTE Group Total Revenue	\$3,910.8	\$7,821.5	\$39,107.7	\$195,538.3
DTE Group Incremental Total Revenue	\$3,025.6	\$6,051.1	\$30,255.6	\$151,278.0
DTE Group Total Margin	\$1,509.6	\$3,019.1	\$15,095.6	\$75,477.8
DTE Group Incremental Total Margin	\$1,212.1	\$2,424.3	\$12,121.3	\$60,606.3
Wellness Share	\$6.0	\$12.0	\$60.0	\$300.0
Implementation Fee (One-Time)	\$150.0	\$150.0	\$150.0	\$150.0
Sales and Marketing Annual Fee	\$180.0	\$180.0	\$180.0	\$180.0
Net Margin Less DTE Fees	\$888.1	\$2,106.3	\$11,851.3	\$60,576.3

Low Estimate Assuming 40%+ Within-Plan Steerage and a Commercial Margin of 28.6%				
All values in thousands of dollars. Modeled With Year 1 Fees and Patient Steerage.				
Metric	500 DTE Covered Lives	1000 DTE Covered Lives	5000 DTE Covered Lives	25000 DTE Covered Lives
Pre-DTE Group Total Revenue	\$885.2	\$1,770.4	\$8,852.1	\$44,260.3
Pre-DTE Group Total Margin	\$297.4	\$594.9	\$2,974.3	\$14,871.4
DTE Group Total Revenue	\$2,664.6	\$5,329.2	\$26,646.2	\$133,231.2
DTE Group Incremental Total Revenue	\$1,779.4	\$3,558.8	\$17,794.2	\$88,971.0
DTE Group Total Margin	\$762.1	\$1,524.2	\$7,620.8	\$38,104.1
DTE Group Incremental Total Margin	\$464.7	\$929.3	\$4,646.5	\$23,232.7
Wellness Share	\$6.0	\$12.0	\$60.0	\$300.0
Implementation Fee (One-Time)	\$150.0	\$150.0	\$150.0	\$150.0
Sales and Marketing Annual Fee	\$180.0	\$180.0	\$180.0	\$180.0
Net Margin Less DTE Fees	\$140.7	\$611.3	\$4,376.5	\$23,202.7

COLLABORATION OF STAKEHOLDERS



Key Benefit Administrators

- Full TPA Administration
- Stop-Loss Administration
- True Transparency & Analytics
- Population Health Management
- Utilization & Case Management
- Sales & Marketing
- Transparent PBM
- Website & Mobile App
- Customizable options & plans



Franciscan Missionaries of Our Lady Health System's DTE

- Branding & Messaging
- Benefit Plan Integration
- Network Steerage
- Direct Relationship with Self-Funded Employers
- Referral Authorization
- Plan Transparency
- Direct Access to Members



Employer

- Reduced Spend
- "At The Table" with Franciscan Missionaries of Our Lady Health System
- Improved Analytics
- Complete Transparency
- Lower Stop-Loss
- Benefit Plan Stabilization
- Healthier Population



Employee

- Reduced Out of Pocket
- Improved Health
- Better Coordination & Navigation of Care
- Direct Access to the Best Medical Providers

ROLES & RESPONSIBILITIES

SERVICES & ADMINISTRATIVE SYSTEMS PROVIDED BY:	
Franciscan Missionaries of Our Lady Health System	KBA Operations
▪ Tier 1 Provider Network ▪ Provider Recruitment – If Necessary ▪ Network Pricing & Contract Terms ▪ Referral Management Protocols & Approvals ▪ Medical Director Oversight ▪ Practice Improvement Program using KBA/AHDI Data with Assistance from KBA	▪ Network Evaluation – Actuarial Pricing ▪ PPO Wrap & Tertiary Transplant Network, if needed ▪ Claims Administration ▪ Plan Design Development, Testing, and Administration ▪ Employer/Member/Provider Customer Service ▪ Customized Member/Provider/Agent/HR Portal ▪ Billing, Enrollment (Electronic/Paper), and Premium Management ▪ Fulfilment – SPD & Customized ID Cards ▪ COBRA/HIPAA Administration ▪ Out of Network Negotiations ▪ Wrap PPO Repricing ▪ Nurse Coaching for Chronically Ill ▪ Nurse Triage to Physicians and Facilities ▪ Stop-Loss Underwriting, Pricing, and Placement ▪ Stop Loss Claims Administration ▪ Complex Case Management ▪ PBM Administration Interface ▪ Reporting • Utilization & Gaps in Care • Clinically Based Physician Profiling • Referrals • Chronic Disease Management ▪ Employer Health Risk Management Reports ▪ Ancillary Administration – Dental, Vision, Weekly Income, 5500, Life, etc. ▪ Health Savings Account (HSA), Flexible Spending (FSA), Health Reimbursement Account (HRA) Administration ▪ Sales & Marketing Plan • Strategy Development: Branding, Brochures, Web Site • Distribution System Development • Sales Management
KBA Consulting	
▪ Program oversight and integration across the enterprise ▪ Goal development, agreement and tracking ▪ Start-up assessments: market, network, operational and readiness ▪ Draft business plan with financial projections ▪ Facilitate program communication and engagement with Franciscan Missionaries of Our Lady Health System departments including system c-suite, operations, HR, managed care, marketing, strategy and business development	



PRE-IMPLEMENTATION

PROVIDER FILE

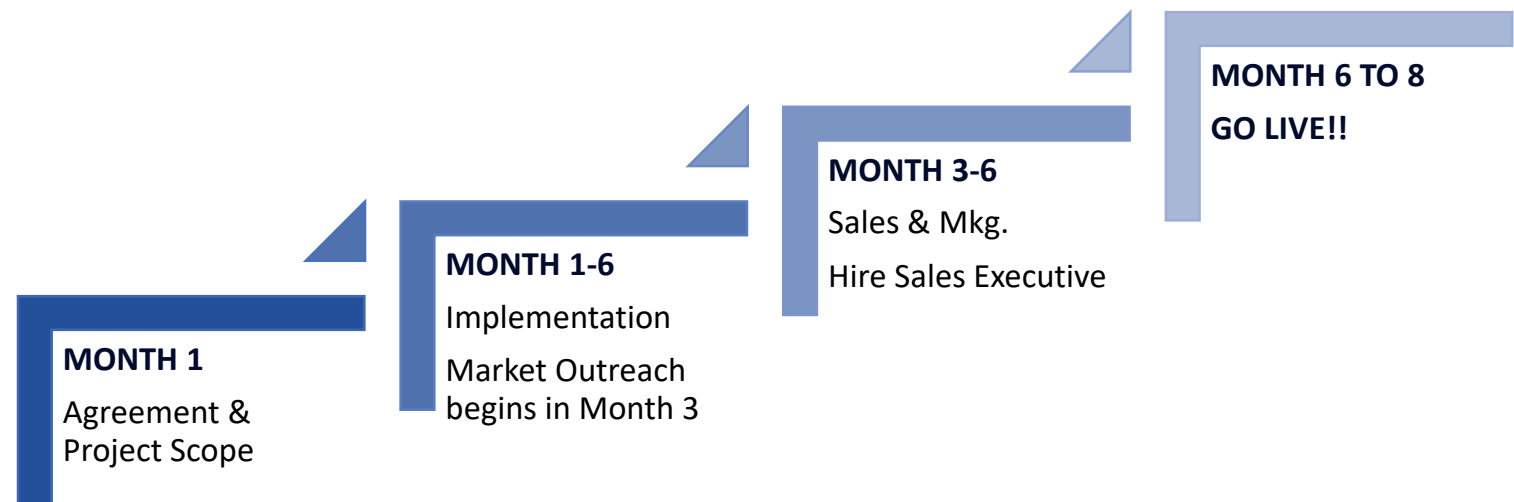
- Determine Gaps in Medical Service Delivery
- Identify Other Key Providers to Potentially Contract with on Behalf of Franciscan Missionaries of Our Lady Health System
- Wrap Network to Cover All Other Provider Types

PRICING

- To Dial in Competitive Stop-Loss Rates and Franciscan Missionaries of Our Lady Health System's Net Revenues/Margins
- Actuarial Evaluation Compared to the Market



IMPLEMENTATION



A PARTNER YOU CAN TRUST



**OUR OBJECTIVES
ARE ALIGNED**



**WE KNOW THE
MARKET**



**COMPLETELY
TRANSPARENT**



**WE HAVE THE
SOPHISTICATION
AND
INFRASTRUCTURE
TO EFFECTIVELY
COMPETE**



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